

Unit 5: Mental & Physical Health

How do psychologists define “normal” behaving, thinking, and acting?

How can we apply health, positive, and clinical psychology principles to change our lives, organizations, and communities in positive ways?

Where can someone find help if they think they or someone they care about is experiencing a psychological illness??

Topic	Learning Objective	Textbook Pages
5.1 Introduction to Health Psychology	Explain how health psychology addresses issues of physical health and wellness as they apply to behavior and mental processes. Explain how stress applies to behavior and mental processes. Explain how reactions to stress apply to behavior and mental processes. Explain how the ways that people cope with stress applies to behavior and mental processes.	637-660
5.2 Positive Psychology	Explain how positive psychology approaches the study of behavior and mental processes. Explain how positive subjective experiences apply to behavior and mental processes.	663-681
5.3 Explaining and Classifying Psychological Disorders	Describe the approaches used to define behaviors and mental processes as psychological disorders. Explain how psychological perspectives define psychological disorders. Explain how interaction models define psychological disorders.	684-695
5.4 Selection of Categories of Psychological Disorders	Describe the symptoms and possible causes of selected: 5.4A- Neurodevelopmental Disorders ☐ Attention-deficit hyperactivity disorder (ADHD) ☐ Autism spectrum disorder (ASD)	737-742
	5.4B- Schizophrenic Spectrum Disorders ☐ Schizophrenia	722-727
	5.4C- Depressive Disorders ☐ Major depressive disorder ☐ Persistent depressive disorder	710-719
	5.4 D- Bipolar Disorders ☐ Bipolar I ☐ Bipolar II	710-711, 713-719
	5.4E- Anxiety Disorders ☐ Specific phobia (& acrophobia and arachnophobia) ☐ Agoraphobia ☐ Panic disorder (& ataque de nervios) ☐ Social anxiety disorder (& taijin kyofusho) ☐ Generalized anxiety disorder (GAD)	699-701, 705-708

5.4 Selection of Categories of Psychological Disorders	Describe the symptoms and possible causes of selected: 5.4F- Obsessive-Compulsive and Related Disorders ☐ Obsessive-compulsive disorder (OCD) ☐ Hoarding disorder	702-703 705-708
	5.4G- Dissociative Disorders ☐ Dissociative amnesia (with and without fugue) ☐ Dissociative identity disorder	730-732
	5.4 H- Trauma and Stressor-Related Disorders ☐ Post-traumatic stress disorder (PTSD)	703-708
	5.4I- Feeding & Eating Disorders ☐ Anorexia nervosa ☐ Bulimia nervosa	735-736
	5.4J- Personality Disorders Cluster A ☐ Paranoid personality disorder ☐ Schizoid personality disorder ☐ Schizotypal personality disorder Cluster B ☐ Antisocial personality disorder ☐ Histrionic personality disorder ☐ Narcissistic personality disorder ☐ Borderline personality disorder Cluster C ☐ Avoidant personality disorder ☐ Dependent personality disorder ☐ Obsessive-compulsive personality disorder	732-735
5.5 Treatment of Psychological Disorders	Describe research in the treatment of psychological disorders.	
	Describe trends in the treatment of psychological disorders.	745-750
	Describe ethical principles in the treatment of psychological disorders.	753-763
	Describe techniques used with psychological therapies.	767-774
	Explain how group therapy is different from individual therapy.	762-763
	Describe effective uses of hypnosis.	N/A
	Describe interventions derived from the biological perspective.	777-789

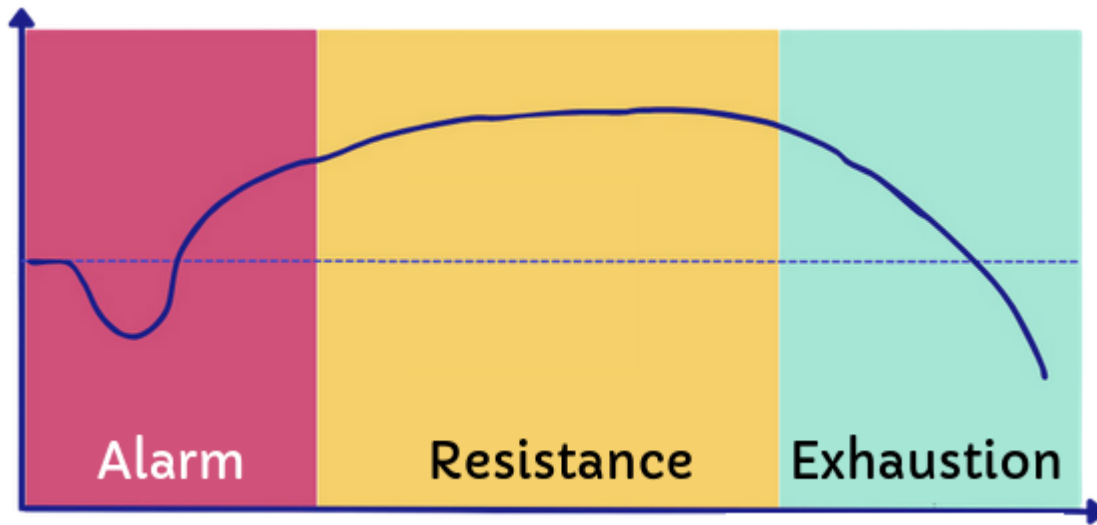
5.1 - Introduction to Health Psychology

Physical Health & Wellness	
Health psychology: a field that explores how biological, psychological, and social factors influence health illness. We take in behaviors, thoughts, and emotions related to health, and works on how individuals and communities can promote well-being and prevent illness. Ex: When you are feeling stressed worried	
Stress: a physiological and psychological response to challenges or demands that exceed an individual's coping abilities. It involves the activation of the body's stress system, and releases hormones like adrenaline and cortisol, which prepare the body to react to perceived threats or pressures.	
Hypertension: aka High blood pressure. It's a chronic medical condition characterized by an abnormally elevated blood pressure in the arteries.	
Immune suppression: the reduction of the body's immune system effectiveness, frequently caused by chronic stress.	
Stressors	
Stressors: Events that we appraise as threatening or challenging	
Eustress:	Distress:
Refers to positive stress that motivates individuals to achieve goals, improve performance, and enhance well-being. It arises from situations perceived as challenging but manageable, stimulating feelings of excitement and satisfaction.	Negative Stress that exceeds an individual's ability to cope effectively, leading to feelings of anxiety, frustration, or overwhelmment. Comes from situations perceived as threatening, harmful, or beyond one's control
Adverse Childhood Experiences (ACEs): Potentially traumatic events occurring before age 18, including things like abuse, neglect, and overall household dysfunction. These can cause toxic stress (distress) and can disrupt brain development causing long-term physical, mental, and behavioral health issues.	

Reactions to Stress

General Adaptation Syndrome (GAS):

Proposed by Hans Selye, describes the body's physiological response to stressors through three stages: Alarm, Resistance, and exhaustion.



Alarm reaction phase: The Body perceives a stressor and activates the flight or flight response. The sympathetic nervous system releases stress hormones like adrenaline and cortisol, preparing the body to respond to the perceived threat.

Resistance phase: This happens if the stressor persists.

- Attempts to adapt and cope with the ongoing challenge. Hormone levels stabilize to sustain the heightened state of readiness, allowing individuals to continue functioning despite the stressor's presence.
- Psychological resources are gradually depleted

Fight-flight-freeze response: an automatic physiological reaction to an event perceived as stressful or frightening.

The body either fights, flights, or freezes.

Exhaustion phase: Physiological resources have become depleted, and the body's ability to cope diminishes.

Chronic stress can lead to mental exhaustion, increased vulnerability to illness, and can impair cognitive function

Tend-and-befriend theory: proposes that in response to stress, individuals, especially women, are biologically predisposed to seek social connections and support (befriending) and nurture others (tending). This theory contrasts with the fight-or-flight response traditionally associated with stress responses.

Coping with Stress	
Problem-focused coping:	Emotion-focused coping:
Actively managing or altering stressful situations through direct actions or problem-solving strategies.	Involves regulating emotional responses to stressors,

5.2 - Positive Psychology

Positive psychology: focuses on the study of human strengths and well-being aiming to understand and promote factors that allow individuals and communities to thrive. - Rather a positive attitude than a negative one
Well-being: Positive emotions such as happiness, contentment, satisfaction with life and overall psychological wellness
Resilience: An individual's ability to adapt and bounce back from adversity or stressful situations, demonstrating mental and emotional strength. It is the capacity to effectively cope with the challenges and maintain well-being.
Positive Subjective Experiences
The Good feelings and thoughts we have about our lives, a core pillar of positive psychology
Gratitude: recognizing and appreciating the positive aspects of life and the kindness of others. This practice can be spontaneous or intentional. Can boost the subject's well being
Signature Strengths/Virtues
An individual's most prominent, authentic, and energizing character traits
Wisdom: Cognitive strengths that involve acquiring and using knowledge - Creativity, Curiosity, Judgement, Love of Learning, Perspective

Courage: Emotional strengths that involve the exercise of will to accomplish goals in the face of opposition.

Humanity: interpersonal strengths that involve tending to and befriending others:

Kindness, Social Intelligence, Love

Justice: Civic Strengths that underlie healthy community life.

- Fairness, Leadership, Teamwork

Temperance: Strengths that protect against excess.

- Forgiveness, Humility, Prudence, Self-Regulation

Transcendence: Strengths that forge connections to the larger universe and provide meaning.

- Appreciation of beauty and excellence, Humor, Hope, Gratitude, and Spirituality

Post-Traumatic Growth

Posttraumatic growth: refers to the positive psychological change experienced as a result of struggling with highly challenging life circumstances. Involve finding a new meaning, personal strength, and appreciation for life following adversity.

- Ex: Survive Cancer -> Change who you are and your outlook on life.

5.3 - Explaining and Classifying Psychological Disorders

Approaches

Psychological Disorders: aka mental disorders, are conditions characterized by abnormal thoughts, feelings, and behaviors. These disorders cause significant distress or impairment in social occupational, or other important areas of functioning.

Its not really a disorder if you aren't constantly doing it

Dysfunction:	Distress:	Deviation from norms:
The <u>extent to which a person's thoughts, feelings, and behaviors, interfere</u> , with their daily life.	The subjective experience of emotional or psychological pain. It is a critical factor in identifying psychological disorders, as individuals may seek help when their distress become unbearable. Suffering, worthlessness, or panic.	Thoughts, feelings, or behaviors that are considered atypical or unacceptable within a society / culture. High levels of deviation can indicate psychological disorders. Phrasing matters when describing things. They don't wanna say rape; they try to dumb it down.

Positive and Negative Consequences of Classifying or Diagnosing Disorders

Diagnosing: a systematic assessment of symptoms, including their duration, intensity, and impact on functioning.

Includes: clinical interviews, psychological testing, and consideration of medical issues.

Dr: person can see and scan broken arm. Its harder for a psychologist to believe or diagnose depression.

Diagnosing has positive and negative consequences, and is subject to bias.

How can the following be consequential to classifying and/or diagnosing a disorder?

Cultural/societal norms	- Behavior Considered abnormal is defined relative to culture context - DSM relies on Western Norms
Stigma	- Fear of being labeled mentally ill discourages people from seeking treatment/diagnosis
Racism	- Black and Latino patients are more likely to be misdiagnosed due to clinician bias

Sexism	- Women are historically over-diagnosed with disorders like depression/anxiety; men are often under-diagnosed
Ageism	- Some symptoms in older people are seen as “normal aging”
Discrimination	- Chronic exposure to discrimination can be a stressor and thus confounding variable.
Culture-bound disorder: disorders specific to certain cultures, often blending physical and psychological symptoms without an underlying organic cause	
Evidence-Based Diagnostic Tools	
Diagnostic and Statistical Manual (DSM): DSM IS NOW ONLINE IS A QUESTION, easier to update	
International Classification of Mental Disorders (ICD): A global health information standard maintained by the WHO (World Health Organization) (THE US NO LONGER FOLLOWS THIS)	
Psychological Perspectives	
Eclectic: Play therapy. Combine elements from psychological theories and techniques to address an individual's specific needs. Play with toys, psychologist asks questions, and gets children to act out what happened using the toys. Using methods from different approaches, such as cognitive behavior therapy (CBT), psychodynamic therapy, humanistic therapy, and others, to make the treatment unique	
How does each of the following perspectives define psychological disorders?	
Behavioral	Views psychological disorders as maladaptive behavior learned through conditioning. Emphasizes observable behaviors rather than internal processes. A psychologist saying I think is poor, a psychologist saying I see, is good.

	A person with arachnophobia might have developed this through classical conditioning. Behavioral therapy, like systemic desensitization, aims to extinguish this fear by gradually exposing the person to the spiders in a controlled and safe environment.
Psychodynamic	- Freudian theory - Focus on unconscious motives, repressed emotions, and early childhood experiences - Disorders arise from unresolved internal conflicts, which can influence behaviors unconsciously. -
Humanistic	School, hopefully used by teachers - Emphasizes personal growth, self-actualization, and achievement of one's potential - Psychological disorders are viewed as barriers to personal growth caused by conditions that thwart self-actualization and reaching true potential, like lack of unconditional positive regard or failure to meet basic needs. Ex: Person with low self-esteem and anxiety might benefit from therapy, because therapist has unconditional positive regard and is empathetic.
Cognitive	Views psychological disorders as a result of faulty or distorted thinking patterns - Cognitive distortions, such as overgeneralization or catastrophic thinking, leading to maladaptive behaviors and emotions - A person with generalized anxiety disorder (GAD) might learn to identify and challenge irrational thoughts that contribute
Evolutionary	Explains psychological disorders in terms of adaptive functions that have become maladaptive in modern environments. - Used to be useful for survival and reproduction - They do not suit a modern environment - Social anxiety or fight or flight were beneficial to early humans, but not so much as now - Treatment focuses on this heightened vigilance and developing modern social skills
Sociocultural	Emphasizes the role of societal and cultural influences on psychological disorders. - Social norms, cultural expectations, and socio-economic conditions as significant contributors to the development and expression of disorders - In Texas: you can say hi to anyone - If you do that in Germany: ur crazy - Water disorders can result from social norms - Body dysmorphia
Biological	Attributes psychological disorders to biological factors such as genetics, neurochemical imbalances, and brain abnormalities - Disorders are seen as medical conditions that can be treated using pharmacological and medical interventions Ex: Schizophrenia is often treated with antipsychotic medications that target dopamine pathways in the brain to reduce symptoms like hallucinations and delusions, alongside supportive psychotherapy to help individuals manage their condition

Interaction Models		
The Biopsychosocial Model		
Biopsychosocial model: Psychological disorders arise from a COMPLEX INTERPLAY of biological, psychological, and social factors. Rejects that mental disorders can be understood SOLELY through one perspective, emphasizing the need to consider MULTIPLE DIMENSIONS of a person's life and health.		
Biological factors	Psychological factors	Sociocultural factors
Genetics, Brain Chemistry, Hormones, Germs	Stress, Trauma, Perceptions, Memory, Personality factors	Context, Norms, Culture, Status, Life Events, Family Dynamic
Diathesis-Stress Model		
Diathesis-Stress Model: suggests that psychological disorders develop due to a COMBINATION of predisposition (diathesis) and environmental stressors.. The diathesis does not show up until a major stress triggers it.		
Diathesis	Stress	
Genetic biological, or psychological, and it predisposes an individual to a disorder, but the disorder only manifests when significant stress triggers it	Environmental factors or life events that can trigger the onset of a disorder when combined with the diathesis. Trauma, Significant life changes, chronic stress, or adverse childhood experiences.	

5.4A - Neurodevelopmental Disorders

Neurodevelopmental disorders: group of conditions that typically manifest early in development, often before the child enters grade school.

Developmental deficits that produce impairments in personal, social, academic, or occupational functioning.

Rangers from specific limitations of learning or control of executive functions to global impairments in social skills or overall intelligence.

3yo that kills pet hamsters, killing animals is a big sign.

Symptoms

Whether the person is exhibiting behaviors appropriate for their age or maturity range

Disorders

Attention-deficit/hyperactivity disorder (ADHD): affects executive functions including planning, organization, and impulse control.

Can lead to academic challenges, social relationship challenges, and lower self-esteem.

Struggles in or avoids tasks that require sustained mental effort

Inattention: Difficulty sustaining attention, failing to follow through on tasks, disorganization, distractability, forgetfulness in daily activities

Hyperactivity: Fidgeting, inability to stay seated, excessive talking, running or climbing in inappropriate situations

Impulsivity: interrupting others, difficulty waiting for a turn, making hasty decisions, without considering consequences.

Autism spectrum disorder (ASD): affects how individuals perceive and interact with the world.

Social communication difficulties -> misunderstandings + social isolation

Restricted interests & Routines -> Stability, Predictability, & Comfort.

Symptoms:

Social Communication Difficulties

Restrictive and repetitive behaviors.

Possible Causes

- **Environmental Factors:** prenatal exposure to toxins, infections, and stress can affect brain development and increase risk of neurodevelopmental disorders.
- **Genetic Factors:** Many neurodevelopmental disorders have a genetic basis, with specific genes/mutations identified as contributing to the development of these disorders
- **Physiological Factors:** Abnormal brain development, including features in brain structure, function, and connectivity is a common feature of neurodevelopmental disorders. Neurotransmitter imbalances and atypical neural circuitry have been implicated in conditions like ADHD & ASD
- **Interactions between Factors:** The development of neurodevelopmental disorders is often the result of a complex interaction between the above three

5.4B - Schizophrenic Spectrum Disorders

Schizophrenic spectrum disorder: Schizophrenia means split mind

- Delusional beliefs, hallucinations, disorganized speech & thoughts, and emotional and behavior

Acute	Chronic
- Active phase described by severe psychosis	- Long term. Lifelong disorder requiring ongoing treatment
Symptoms	
Positive: excessive or distorted thoughts and behaviors Added to personality	Negative Removed from personality
- Hallucinations, Delusions, Disorganized Thinking/Speech, Disorganized motor ability	- Avolition/Lack of Motivation - Affective Flattening(Reduced emotional expression) - Reduced Speech - Reduced Pleasel
Delusions	
Delusions: A bizarre of far-fetched beliefs that continues in spite of competing and contradictory evidence	
Delusions of persecution:	Delusions of grandeur:
People believe that spies, aliens, or evening neighbors are plotting to harm them	People believe they are someone very powerful or important
Hallucinations	
Hallucinations: a false or distorted perception that seems vividly real. Hallucinations can be visual, or even olfactory(smell), people often report hearing voices that command them	
Disorganized Thinking or Speech	
Disorganized thinking or speech: WORD SALAD: Artificial words and jumping together, lack of contact with reality	

Word salad: artificing and jumbling of words without any real meaning.

Disorganized Motor Behavior

Disorganized motor behavior: (Catatonia)

-Involuntary movement or motor behavior that is unusual, unpredictable, or purposeless

Catatonia: Treatable neuropsychiatric syndrome characterized by abnormal movement, behavior, and responsiveness

Catatonic excitement:

Positive Catatonia: Excitement such as shaking the head to remove unwanted thoughts

Catatonic stupor:

Negative: Aka Stupor, assuming an immobile stance for an extended period of time.

Negative Symptoms

Negative symptoms: taken away from personality

Flatt affect:

A Flattening of speech and the emotions

Catatonic stupor:

Lack of movement what so ever

Possible Causes

Genetic: the risk of developing schizophrenia increases if it is already in the family. Biological parent w/ schizophrenia -> more likely for child to develop it.

Biological: Brain structure w/ enlarged fluid-filled ventricles of the brain, Paternal age is positive correlated with schizophrenic risk, auto-immune disorders increase changes

Diathesis-Stress: people inherit predispositions(diathesis), which increase their schizophrenic risk, then stress triggers the episodes

Environment: Viruses during pregnancy may increase risk of brain abnormalities.

Dopamine hypothesis: Overactivity of certain dopamine neurons may cause some forms of schizophrenia, especially involving hallucinations and delusions.

Antipsychotic medications block the excess dopamine and reduce the hallucinations and delusions.

5.4C - Depressive Disorders

Depressive disorders: the presence of sad, empty, or irritable mood along with physical and cognitive changes that affect a person's ability to function

Symptoms

- **Major Depressive Disorder (MDD):** is a characterized by a persistent and pervasive low mood, and with a range of emotional and physical symptoms. You need to experience at least 5 of these symptoms for at least 2 weeks, with one of them being depressed mood/ loss of interest or pleasure
- **Depressed mood:** persistent feelings of sadness, emptiness, or hopelessness.
- **Changes in weight or appetite:** SIGNIFICANT loss OR gain
- **Sleep Disturbances:** Insomnia or Hypersomnia
- **Fatigue:** persistent feelings of tiredness or lack of energy
- **Feelings of worthlessness**
- **Cognitive Impairment:** cannot concentrate, making decisions, or thinking clearly
- **Psychomotor Agitation:** Restlessness or slowed movements and speech
- **Suicidal Ideation:** Recurrent thoughts of death, suicidal thoughts, or suicide attempts.

Disorders

Persistent depressive disorder: Depressed mood for at least two years in adults or one year in children/adolescents

Chronic Depressed Mood: A low, dark, sad mood that persists over time

Appetite & Weight Changes: poor appetite or overeating

Sleep Disturbances: Insomnia or excessive sleeping

Fatigue: Persistent lack of energy

Low Self-Esteem: Feelings of inadequacy or low self worth.

Difficulty Concentrating: Problems with focusing, making decisions, or remembering things

Hopelessness: Pervasive feelings of hopelessness about the future.

- The difference between this and MDD is a psychologist

Learned Helplessness:

- Series of negative outcomes or stressors causes someone to believe that the outcomes are out of one's control.
- Ex: When exposed to repeated shocks that they could not control, dogs refrained from taking action when they could prevent the shocks.

Possible Causes

MDD

- **Biological:**
 - Neuroticism: a well established risk factor (Negative affectivity)
- **Genetic**
 - First Degree Family Member
- **Behavioral**
 - Learned Helplessness
- **Social:**
 - ACES(Adverse Childhood experience)
- **Cultural**
 - Gender
- **Cognitive:**
 - Maladaptive thoughts
 - Cognitive Triad (depressive stems from automatic, negative thinking patterns about three areas: the self, the world, and the future)

5.4D - Bipolar Disorders

Bipolar disorders: Mood disorders characterized by extreme mood swings between depression and mania (or hypomania)

Symptoms

Depression:	Mania:
- Persistent sad, empty, or hopeless mood - Loss of interest or pleasure - Fatigue and loss of energy - Changes in sleep (insomnia or hypersomnia)	- Abnormally elevated, expansive, or irritable mood - Decreased need for sleep - Inflated self-esteem or grandiosity

Cycling: Pattern of shifting between mood episodes

- Rapid Cycling can occur
- Episodes can be separated by periods of normality
- Triggered or worsened by stress, substance use, sleep deprivation, or antidepressant use without a mood stabilizer

Disorders

Bipolar I disorder: At least one full manic episode lasting 7+ days

- Severe enough to cause marked impairment in social/occupational functioning
- Depressive episodes are common but not required

Bipolar II disorder:

- At least one hypomanic episode AND one major depressive episode
- **Hypomania:** less severe form of mania - elevated mood/energy that is noticeable but does not cause severe impairment

Possible Causes

- Genetic: Strong hereditary first degree relatives
- Neurobiological: Dysregulation of neurotransmitters can cause
- Circadian Rhythm Disruption
- Psychological factors: High Stress, Trauma, and negative cognitive patterns
- Environmental triggers like substance abuse, major life changes, and sleep disruption

5.4E - Anxiety Disorders

Anxiety disorders: are characterized by excessive fear and/or anxiety with related disturbances to behavior.

Symptoms

- Can very exceptionally dramatically
- Excessive and persistent worry/or that interferes with daily functioning
 - Both Psychological and physiological manifestations

Disorders**Specific Phobia**

Specific Phobia: intense irrational fear of specific object or situation that is typically disproportionate to the actual threat.

Intense Fear: immediate and overwhelming fear when encountering the specific object or situation.

Acrophobia:

Fear of heights

Arachnophobia:

Spiders

Agoraphobia

Agoraphobia: an intense fear of being in situations where escape might be difficult or help might not be available in the event of a panic attack or other incapacitating symptoms.

Fear of Open or crowded spaces

Panic Disorder

Panic Disorder: unexpected panic attacks which are sudden periods of intense fear or discomfort that peak within minutes.

Ataque de nervios: "Attack of the nerves" a culturally specific syndrome primarily in Latin American and Caribbean communities characterized by the sudden onset of intense emotional distress and physical symptoms, often in response to stressful or traumatic events.

Social Anxiety Disorder

Social Anxiety Disorder: an intense fear of social situations where the individual may be exposed to potential scrutiny or embarrassment

Taijin kyofusho: Type of anxiety specific to Japanese culture characterized by intense fear of offending or embarrassing others through one's appearance or behavior.

Generalized Anxiety Disorder (GAD)

Generalized Anxiety Disorder (GAD): excessive unconditional worry about various aspects of life such as work, health, or social interactions occurring more days than not for at least six months.

Possible Causes

Behavior: learned associations

Cognitive: Maladaptive thinking/emotional response

Biogenic: 1/3 of causes are just from genetics

Neuroticism: a fundamental personality trait characterized by an enduring tendency to experience negative emotions, such as anxiety, depression, anger, and emotional instability

5.4F - Obsessive-Compulsive and Related Disorders

Obsessive-compulsive and related disorders: (OCD) - The presence of obsessions, compulsions, or both	
Symptoms	
Obsession:	Compulsion:
Persistent, intrusive thoughts, images, or urges that cause significant anxiety or distress. Ex: Fears of contamination, harming others, or fears of making mistakes	Repetitive behaviors or mental acts performed in response to an obsession or according to rigid rules - Intended to reduce anxiety or prevent a feared event - Ex: Hand washing, checking, counting, and repeating actions
Disorders	
Obsessive-compulsive disorder: Recurring intrusive thoughts(obsessions) that prompt the performance of of neutralizing rituals (compulsions) - Obsessions and compulsions are time consuming - Symptoms cause significant emotional distress and social impairment - Individuals may avoid certain situations or objects that trigger their obsessions or require them to perform compulsions	
Hoarding disorder: a mental health condition characterized by persistent difficulty discarding or parting with possessions, regardless of their value. - Excessive accumulation Difficulty Discarding items Emotional Distress Functional Impairment Indifference or denial to the disorganization of their lives and its impact on relationships and functions	
Possible Causes	
- Temperamental: Greater internalizing symptoms, higher negative emotionality, and behavior inhibition in childhood - Environmental: Perinatal events, premature birth, maternal tobacco, and other events during birth/pregnancy - Genetic and Physiological: First Degree Relatives - Culture: OCD is global, but regional variation in symptoms exist. - Men have an earlier onset than woman - Learned Associations - people develop connections between stimuli, formulating the disorder - Maladaptive THinking - Biological and Genetic Factors	

5.4G - Dissociative Disorders

Dissociative disorders: Disorders of consciousness in which a person appears to experience a sudden loss of memory or a change of identity.
Symptoms

- **Memory Disruptions**
- **Conscious Disruptions**
- **Identity Changes**
- **Perception Differences**

Disorders

Dissociative amnesia: characterized by a partial or total inability to recall apst experiences and important information. This can be in response to atraumatic event

What is **fugue** and is it required to be present to diagnose someone with dissociative amnesia?

Suddenly and inexplicably leaving home and taking on a new identity with no memory of a former life. Person experiences amnesia but is otherwise normally functioning. It is not required, but it is a strong indicator

Dissociative identity disorder: (Multiple Personality Disorder): The presence of multiple distinct personalities in the same individual. Each personality has its own name, self image, memories, and behaviors.

Possible Causes

Experience of severe trauma or stress, like adverse childhood experiences

- Early disruptions to attachment relationships can contribute.

5.4H - Trauma- and Stressor-Related Disorders

Trauma- and stressor-related disorders: Result from exposure to traumatic events	
Symptoms	
- Hypervigilance - Severe anxiety - Flashbacks to traumatic or stressful experiences - Imsonia - Emotional Detachment - Hostility	
Disorders	
Posttraumatic stress disorder (PTSD): ASD symptoms persist for more than a month after a traumatic event - Feelings of anxiety, horror, and helplessness after experiencing a traumatic event - People experience flashbacks, nightmares, depression, uncontrollable crying, irritability, and an inability to concentrate and maintain relationships.	
Possible Causes	
- Trauma or stress - Genetics can affect, 1st degree relatives show a correlation - Temperament can also have an affect, people are more predisposed to things like this (Diathresis- Stress) - Pre-existing mental health conditions, previous trauma - Coping Mechanisms	

5.4I - Feeding and Eating Disorders

Feeding and eating disorders: Characterized by altered consumption or absorption of food that impairs both health and psychological functioning	
Symptoms	
Lack of appetite - Dramtic Weight Loss - Uspet Stomach - Abdmominal Pain - Gastroinstestionalk issues - Picky Eating - Dramatic Weight Gain	
Disorders	
Anorexia nervosa: - Life threatening disorder that involves intense fear of weight gain or becoming overweight - Distorted perception of one's own weight/shape (body dysmorphia) - Restriction of caloric intake - Affects more females than males - Risk of Suicide is also higher	

Bulimia nervosa: - Potential life threatening - Secretive bingeing (large food in little time) - Followed quickly by some form of compensatory behavior such as purging(self induced vomiting, laxatives, fasting, or excessive exercise)
Possible Causes
- Biological: Abnormalities in brain structure and function (eg hypothalamic disruption), hormonal imbalances, neurotransmitter irregularity - Genetic: Hereditary association w/ siblings and relatives who develop the disorders - Behavioral: Chronic Eating, Restrictive eating, excessive exercise - Cognitive: Maladaptive thinking about food, weight, body images, heightened perfectionism, control, self-criticism, and false binaries(logical fallacies)

5.4J - Personality Disorders

Personality disorders: A group of mental health conditions characterized by enduring patterns of behavior, cognition, and inner experience that deviate markedly from cultural expectations. These patterns are inflexible, pervasive, and stable over time, leading to significant distress or impairment in personal, social, or occupational functioning		
Symptoms		
Enduring patterns of internal experience and behavior that is deviant from one's culture; is pervasive and inflexible; begins in adolescence or early adulthood; is stable over time; and leads to personal distress or impairment.		
Disorders		
Cluster A		
Erratic Behaviors characterized by odd or eccentric behaviors. They include paranoid personality disorder, Schizoid Personality Disorder, and Schizotypal Personality Disorder		
Paranoid personality disorder:	Schizoid personality disorder:	Schizotypal personality disorder:
Distrust , perverse and unjustified mistrust, and suspicion of others. People with PDD are often preoccupied with doubts about the loyalty or trustworthiness of others and interpret benign or ambiguous actions as threatening or demeaning. Symptoms: persistent doubts about the intentions of others, including fears of being exploited or deceived. Misinterpretation, Reluctance to Confide, Defensiveness, and Grudges.	Detachment from social relationships. Restricted range of emotional expression. Pervasive detachment from social relationships and a restricted range of emotional expression.	Eccentric behavior, unusual beliefs or magical thinking, and social anxiety
Cluster B		

Cluster B personality disorders are characterized by dramatic, emotional, or erratic behaviors. The main disorders in this cluster include AntiSocial Personality Disorder, Histrionic Personality Disorder, Narcissistic Personality Disorder, and Borderline Personality Disorder.

Dramatic, Emotional, Erratic

Antisocial personality disorder:		Histrionic personality disorder:	
*includes problems also known as psychopathy and sociopathy - Disregarding and violating the rights of others that includes such problems as deceitfulness, impulsivity, aggressive behavior, recklessness, lack of consciousness - Irresponsibility, others = prey		Characterized by a pervasive pattern of excessive emotionality and attention-seeking behavior. Dramatic, flamboyant, and theatrical behaviors to gain attention and approval.	
Cluster B con't.			
Narcissistic personality disorder:		Borderline personality disorder:	
Grandiosity, Admiration Pervasive pattern of grandiosity, need for admiration, and lack of empathy. Individuals with NPD often have an inflated sense of self, and feel the need to be admired.		Instability of interpersonal relationships, self-image - Pervasive pattern of instability, interpersonal relationships, self-image, and affect, along with marked impulsivity. - Experience emotional swings and have trouble maintaining stable relationships. Impulsivity, Unstable Relationships, unstable self-image This is fact Vertical along wrist equal suicide, Horizontal is cry for help	
Cluster C			
Anxious, Fearful - Characterized by anxious and fearful behaviors. These disorders involve patterns of thinking and behaving that are pervasive, inflexible, and maladaptive, leading to significant distress or impairment.			
The three primary disorders in this cluster are avoidant personality disorders			
Avoidant personality disorder:	Dependent personality disorder:	Obsessive-compulsive personality disorder:	

Social Inhibition, Feelings of inadequacy, Hypersensitivity to negative evaluation. Pervasive pattern of social inhibition, feelings of inadequacy, and hypersensitivity to negative emotions. People avoid social interactions and situations due to intense fear of criticism, rejection, or disapproval.	Submissive and clinging behavior, Excessive need to be taken care of Characterized by a pervasive and excessive need to be taken care of, leading to submissive and clinging behaviors and fears separation.	Characterized by a preoccupation with orderliness, perfectionism, and control, often at the expense of flexibility.
Possible Causes		
Biological/Genetic: Genetic predisposition, Frontal Lobe, Autonomic Nervous System Social/Cultural: Trauma, Family relationships Behavioral: Conditioning, Failure to learn from consequences, modeling Cognitive: Maladaptive thinking		

5.5AB - Personality Disorders Research & Trends in Treatment of Psychological Disorders

Research
Psychotherapy: Psychiatrist/Psychologist/Therapists talks with you but also gives you things to do outside of those sessions A collaborative, confidential treatment where a trained professional helps individuals identify and change unhealthy emotions, thoughts, and behaviors.
Evidence-based interventions: Treatment providers use varied approaches to help treat psychological conditions
Cultural humility: an approach in therapy that emphasizes an ongoing process of self-reflection and learning about cultural identities of clients The therapist needs to recognize their own culture's biases and limitations and help conform to and understand the client's cultural background, values, and experiences

Therapeutic alliance: The collaborative and trusting relationship between a therapist and a client.

Therapist will tell someone else when:

- You're going to hurt yourself
- You're going to hurt someone else
- You're being hurt

Trends

Psychotropic medication: This medicine has been getting better, but more expensive over the years.

Deinstitutionalization: 20th Century: Increased effectiveness of psychotropic medication therapy, hospitals and asylums deinstitutionalized (removed from the institution) massive numbers of people

- But a lot of them weren't fully recovered
- Some of them can't afford the medicine
- Turn to Drugs, end up homeless or in prison
- In the 70s, the US starting taking away insurance
- Signed deal with China, companies moved to China, less benefits -> more people homeless

5.5C - Ethics in the Treatment of Psychological Disorders

According to the APA, your therapist should follow the following:

Nonmaleficence: Seek to do no harm

Beneficence: Seek to benefit you

Fidelity: A feeling of trust and a defined role as a therapist to uphold a professional standard of conduct, and be of service to the therapeutic community

Integrity: be honest, truthful, and accurate.

Respect for people's rights and dignity: Respect the dignity and worth of you and others, recognizing the right to privacy, confidentiality, and self-determination.

5.5D - Psychological Therapy Techniques

Psychodynamic Therapies

- Rooted in Freudian psychoanalytic theory
- Aims to explore unconscious processes and how they influence current behavior.
- Emphasizes the role of early childhood experience and internal conflicts (psychodynamic perspective)
- Goal is to bring unconscious thoughts and feelings to conscious awareness, allowing the individual to gain awareness of and resolve these conflicts

Techniques

Free association: a psychoanalytic technique, developed by Sigmund Freud, where patients express thoughts, words, or images exactly as they come to mind without censorship

Bypasses conscious defenses to uncover unconscious conflicts, memories, and emotions,

Dream interpretation: Analyzes the content of dreams to uncover unconscious desires, fears, and conflicts

- Dreams = window into the unconscious mind, revealing hidden aspects of the psyche

Cognitive Therapies

- A form of psychotherapy developed by Aaron T. Beck. It focuses on identifying and changing distorted or unhelpful thinking patterns, beliefs, and attitudes that contribute to emotional distress and maladaptive behaviors.
- The idea is that dysfunctional thinking leads to negative emotions and behaviors, and by altering these thoughts, individuals can achieve more positive outcomes. A person suffering from depression might believe they are worthless.
- Cognitive therapy allows them to learn and identify this negative thought, look for evidence against it, and replace it

Techniques

Cognitive restructuring:

A core component of Cognitive Therapy, involving techniques that help individuals identify, challenge, and modify distorted thoughts.

- The process is essentially recognizing these cognitive distortions, then validating them, and then using that validation to undertake a more realistic understanding and way of thinking.

Fear hierarchy: A fear hierarchy is used in exposure therapy to treat anxiety disorders, like phobias and PTSD, which is a graded list of anxiety provoking situations/stimuli. Clients will work their way up the hierarchy in a controlled setting

Maladaptive thinking: negative and irrational thoughts or beliefs that contribute to unhealthy behaviors and emotional distress.

- Based on cognitive distortions such as overgeneralization or black and white thinking

Cognitive triad: a concept in cognitive therapy that describes three types of negative thoughts that are common in depression: negative views about the self, the world, and the future.

- These negative thoughts reinforce each other in a triangle

Applied Behavioral Analysis

- A therapeutic approach based on the principles of behaviorism. It focuses on improving specific behaviors, such as social skills, communication, reading, and adaptive learning skills, through the application of behavioral principles

Exposure therapy: a set of techniques designed to help individuals confront and reduce their fear and anxiety by gradually exposing them to the feared object or situation in a controlled manner

Systematic desensitization: behavioral therapy technique used to treat phobias and anxieties by pairing deep relaxation with gradual exposure

Aversion therapy: behavioral treatment that reduces unwanted habits by pairing them with unpleasant or painful stimuli

Token economy: type of operant conditioning used in therapeutic settings, particularly in institutions or schools, where individuals earn tokens for engaging in desired behaviors

Tokens -> Rewards

Ex: Class Dojo

Biofeedback: A technique that teaches individuals to control psychological processes such as heart rate, muscle tension, and brain wave activity via real time feedback

- The goal is to gain voluntary control over these to improve physical and mental health.

Cognitive Behavioral Therapies

CBT is a widely used, evidence-based therapeutic approach that aims to change maladaptive thinking patterns and behaviors. - Cognitive Distortions -> EMotional Distress - SOLution = Fix cognitive distortions
Dialectical behavior therapy: Type of therapy designed for individuals with borderline personality disorder, uses CBT techniques with mindfulness techniques with principles of acceptance and change
Rational-emotive behavior therapy: Form of therapy developed by Albert Ellis - Focuses on identifying and changing irrational beliefs that lead to emotional and behavioral issues. - Emphasizes rational thinking
Humanistic Therapies
- Focuses on the individual's subjective experience and the belief that people possess free will and the potential for personal growth, - Allows people to understand themselves and achieve their full potential
Active listening: A communication technique used in therapy to ensure that the therapist understands the client's message, Involves paying close attention to clients words, reflecting, and providing feedback
Unconditional positive regard: A fundamental humanistic concept, particularly in Carl Rogers approach, therapist accepting and valuing the client without judgement or conditions Client-centered (person-centered) therapy: one of the most widely used models in psychotherapy today. It involves creating a comfortable, non-judgmental environment by demonstrating empathy, genuineness, and

Additional Therapeutic Pathways

Group therapy:	Individual therapy:
A small group of individuals who meet regularly with one or more therapists to discuss their issues, share experiences and support each other.	A one-on-one treatment where a trained profession interacts directly with a client.
Hypnosis:	
A psychological state characterized by heightened suggestibility, focused attention, and deep relaxation. During hypnosis,m individuals are guided into a trance-like state by a trained practitioner, often through verbal suggestions and relaxation techniques. The patient may experience heightened relaxation, focus,and can help reduce pain	

5.5G - Biological Interventions in Treatment

Psychoactive Medications	
Psychoactive medications: interaction with specific neurotransmitters in the central nervous system to address possible biochemical causes	
Antidepressants:	Antianxiety drugs:
Designed to treat depression by inhibiting the reuptake of serotonin(regulates mood)	Reduces anxiety and produces relaxation by lowering sympathetic activity in the brain
Lithium:	Antipsychotic medications:
Mood Stabilizing drug that is used to treat the combination of manic episodes and depression characteristics of bipolar disorder	Designed to diminish or eliminate positive symptoms of schizophrenia like hallucinations and delusions. Decrease dopamine receptors. Haldol and Thiorazine are best known.
Side Effects	
Tardive dyskinesia: serious, often irreversible side effect of long-term antipsychotic medication use - Repetitive movements, like lip-smacking, tongue protrusion, and rapid blinking - Arises from excessive dopamine receptor sensitivity in the brain	
Surgical and Invasive Interventions	
Psychosurgery: the most dramatic and least used intervention, involves removing or lesioning brain tissue, This is the last resort	
Lesioning: Refers to damaging brain tissue, Often used in experiments, Precise MRI guided surgery to destroy specific nerve clusters - Often used on patients with severe seizures, depressive disorder or OCD	
TMS (transcranial magnetic stimulation): An alternative to ECT that involves pulsating a magnetic coil over the prefrontal regions of the brain, treats depression with minimal side effects	
Electroconvulsive therapy: Used to treat serious depression. Works faster than drugs, used for suicidal places. Place two electrodes on the outside of the patient's head and passing 100Volts through the brain.	
Lobotomy: an archaic practice used to treat severe mental illness by severing nerve fibers connecting the thalamus to the frontal lobes through the eye socket	

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